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A CROSS-CULTURAL STUDY ON ATTITUDES TOWARDS MENTAL ILLNESS AND WILLINGNESS TO SEEK PROFESSIONAL HELP AMONG JAPANESE AND U.S. COLLEGE STUDENTS

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Abstract – This study explored predictors of willingness to seek help for mental illness. We hypothesized that mental illness stigma, internal locus of control, and lower SES would be associated with less likelihood of seeking professional help for mental illnesses. We also hypothesized that these results would differ in two samples based on culture (U.S. versus Japanese). College students in each country completed an online survey in their language. Results showed partial support for Hypothesis 1 in that help seeking was more likely in individuals with lower mental health stigma. However, help seeking was surprisingly correlated with higher external locus of control and was not significantly correlated with SES. Exploratory analyses also showed that women were more likely to seek help than men. Supporting Hypothesis 2, these results differed by country; for the Japanese sample, the only significant predictor of help seeking was mental health stigma. These findings are similar to previous studies which highlight the importance of mental health stigma regarding willingness to seek help. Future research can build upon this study's foundation by further exploring the questions and patterns of results found here.

Taking care of our mental health and well-being is a crucial part of living well; it also allows us to better cope with the stresses of life. According to the World Health Organization (World Health Organization, 2018), mental health is "a state of well-being in which an individual realizes his or her own abilities, can cope with the normal stresses of life, can work productively and is able to make a contribution to his or her community." Intrapersonal relationships, biological factors, life experiences such as trauma or abuse, family history of mental health problems, and a variety of other circumstances may all negatively influence people's mental health (MentalHealth.gov, 2022).

Taking care of one's mental health ensures that one's capacity to enjoy life is retained and allows individuals to cope with life's difficulties and to make meaningful contributions to their communities (MentalHealth.gov, 2022). Unfortunately, some people neglect care of their mental health due to stigma. Mental health stigmatization is a problem all over the world (Corrigan, 2004; Pescosolido et al., 2008). The purpose of this study was to explore variables that might affect degrees of mental health stigma (such as locus of control and socioeconomic status), as well as to compare attitudes toward mental health and help seeking between two cultures: Japan and the United States.

Attitude Towards Mental Illness

According to HealthyPeople.gov (2020), "Mental disorders are among the most common causes of disability. The resulting disease burden of mental illness is among the highest of all diseases. In any given year, an estimated 18.1% (43.6 million) of U.S. adults ages 18 years or older suffered from any mental illness and 4.2% (9.8 million) suffered from a seriously debilitating mental illness." Despite the pervasive nature of mental illness, the World Health Organization revealed that stigma, discrimination, and neglect of mental health all contributed to prevent care and treatment for people affected by mental illness (World Health Organization, 2018). The reduction of the public health burden of mental illness begins with addressing stigma. Mental health stigma is not just a problem in the United States. For example, participants in a Japanese survey on public

attitudes of mental illness revealed the idea that mental disorders are treatable, yet stigma towards mental illness remained rather prevalent (Kasahara-Kiritani et al., 2018).

There are many factors which may mediate or moderate individual attitudes toward mental illness; this study focused on four: stigma, locus of control, socioeconomic status (SES), and culture. These factors were the predictor variables in this study, with willingness to seek professional help as the outcome variable. Stigma, locus of control, and SES can be more fully understood with a cultural lens; we focused on comparisons between the U.S. and Japan.

Stigma

Stigmatization of people with mental illnesses is a major problem all over the world; this stigma can discourage people with mental illnesses from seeking help (Corrigan, 2004; Pescosolido et al., 2008). That said, recent research indicated a considerable reduction (at least 10%) in public stigma toward depression in the United States over the last 25 years, likely due to changing cultural norms in recent generations (Pescosolido et al., 2021). In short, older generations have more conservative views about mental illness, while millennial generations are more progressive. This trend has been occurring in the U.S. for quite some time, with positive change being documented in studies comparing generational perceptions starting in 1950 (Phelan et al., 2000).

In comparison to other industrialized countries, there is evidence that stigma-related attitudes toward people with mental illness remain quite high in Japan (Ando et al., 2013; Follmer & Jones, 2018; Griffiths et al., 2006; Kudva et al., 2020; Naganuma et al., 2006; Zhang et al., 2020). Around 80% of people in Japan diagnosed with a mental condition based on the Diagnostic and Statistical Manual of Mental Disorders, Fourth Edition (DSM-IV) did not obtain any mental health care in the previous year (Naganuma et al., 2006). In some Asian cultures (including Japan), psychological suffering is generally considered a diagnosis that connotes personal weakness and is therefore often concealed from others (Munakata, 1986; Ng, 1997). The “weakness” might be associated with psychological faults (such as lack of resolve or low self-control) or sociomoral issues (such as a breakdown of family structure). Historically, patients and family members faced social prejudice in areas such as marriage, business, and education (Samuma, 1978).

While the research cited above found commonalities across several Asian nations in terms of culture, other scholars note differences. For example, five studies comparing mental health stigma in Japan to other nations (such as China and Taiwan) found varying degrees of stigma—with Japan in the middle range (Griffith et al., 2006; Haraguchi et al., 2009; Jorm et al., 2005; Kurumatani et al., 2004; Nakane et al., 2005). The variation was moderated by several factors such as whether medical and psychological care are socialized, providing easier access to general practitioners or social workers. Stigma within a given nation may also be tied to average education level of citizens, including specifically health literacy education (Ishige & Hayashi, 2005).

Furthermore, the unique history and essence of Japanese society may explain why Japanese people have frequently displayed stigmatizing attitudes toward mental illness. According to Furnham and Murao (2000), it was once considered taboo to discuss mental illness in public, and until recently family members were responsible for the care of those with mental illnesses. Japanese people have historically been reticent to reveal their genuine thoughts in public, preferring to present beliefs and actions that will be accepted by the public majority. They generally prefer not to stick out as being different from others, valuing conformity to the group over individual needs (Naito, 1992; Rasiban & Hardianti, 2022; Schmidt-Petri et al., 2022). Japanese culture also highly values personal control and willpower, and mental health symptoms can be viewed as a lack of mental self-control (Munakata, 1986). If a Japanese person is perceived as losing this willpower, they are socially trained to feel ashamed. Despite the fact that many mental health issues have solutions (e.g., prescription medications), over two-thirds of those who suffer in Japan never seek help from a health professional (Desapriya, 2002). These differences between Japanese and U.S. culture led to the current study’s comparison between the two nations, to provide further data regarding the potential effect of culture on attitudes toward help seeking.

Locus of Control

Locus of control refers to how much an individual feels that outcomes in their life are dependent on their own choices and actions (Rotter, 1954, 1966). While typically measured along a range, scores are often categorized as simply “internal” or “external.” Individuals with an internal locus of control hold the belief that outcomes are determined by their own actions or personal attributes. Individuals with an external locus of control think that life events are governed by elements beyond their control, such as luck or chance. Several studies (e.g., Shute et al., 1984; Skinner et al., 1998; Weisz & Stipek, 1982) have found that locus of control trends toward internal with age and maturity. Development of locus of control is also connected to

family style and resources, cultural stability, and experiences with effort resulting in reward. In the U.S., many individuals with an internal locus of control were raised in households that emphasized values that align with an internal locus. These families put a priority on work, education, responsibility, and critical thinking. According to Schultz and Schultz (2005), children in the U.S. who grow up in households where their parents are supportive and consistent in punishments develop an internal locus of control. Alternatively, children in the U.S. whose parents have an external locus of control are more prone to ascribe their triumphs and failures to external factors, possibly passing these beliefs on to the next generation (Schneewind, 1955).

In addition to family influence, one's locus of control might be influenced by national culture. According to cross-cultural studies (Fukuzawa & Inamasu, 2020; Hsieh et al., 1969; Morren & Grinstein, 2016), Americans may be more internally oriented than Asians in general. Gillin (1955) has commented on American society's focus on personal productivity, pragmatic ingenuity, independence, and self-reliance. Hsu (1961) emphasized the characteristics of individuality, independence, and self-reliance that are often regarded as traits of the American character. Hsu, on the other hand, characterized Asians (in particular, Chinese people in this study) as believing in luck, chance, and fate significantly more than Americans. There is some evidence to suggest that this is also true of Japanese people. According to Caudill and DeVos (1956), Japanese people are more tightly governed by external constraints than by internal control. According to McGinnies et al. (1974), the fact that Japanese respondents are more disposed to subscribe to external control appears congruent with the structure of Japanese society, where there appears to be less potential for social and professional mobility.

A study on locus of control and its influence on subjective well-being revealed that higher levels of externality result in lower levels of well-being (April et al., 2012). This finding builds on prior research (Morrison et al., 1994), which revealed that belief in personal control (or internal locus of control) predicted perceiving mental patients as comparable to "normal" people, differing only in degree but not in kind. Internal locus of control appears to be associated with higher well-being as well as lesser stigmatization of others who have been diagnosed with a mental illness.

When it comes to help seeking for mental illness, two hypotheses seemed potentially valid. On one hand, people with an internal locus of control might perceive that mental illness is controllable with enough willpower and discipline; therefore, seeking outside help would not be needed. On the other hand, people with an internal locus of control have higher well-being and lower stigma according to past research (April et al., 2012), both of which might lead to more help seeking for mental illness. In this study we hypothesized the former (higher internal locus would be associated with less help seeking), due to the emphasis on values such as independence and self-reliance in people with high levels of internal locus of control.

Socioeconomic Status

Socioeconomic status (SES) is described as an individual's position on a social-economic scale that takes into account factors such as education, income, occupation, location of residence, and, in certain communities, heritage and religion (APA, 2022). A number of surveys (Bhavsar et al., 2014; Cechnicki et al., 2011; Corrigan & Watson, 2007; Hansson et al., 2016; Robinson & Henderson, 2018; Wang et al., 2007) have discovered that those with lower SES hold more stigmatizing opinions on mental health. This may intensify the experience of mental illness for people in these groups, who also have a greater prevalence of mental illness (McManus et al., 2016). According to a number of studies, treatment response differs by SES level. Cohen et al. (2008), for example, found that inhabitants of middle- and high-income census tracts were more likely to react to late-life depression therapy and less likely to express suicidal ideation than residents of low-income census tracts. Jakubovski and Bloch (2014) discovered that other markers of low socioeconomic position, such as low income, education, and unemployment predicted poor responses to treatment for major depressive disorder.

The relationship between SES and mental health stigma is unclear, however, and may be further complicated by nation and/or culture. A research study conducted inside a Buraku area of Nishinari ward in Japan—known as a lower SES region—discovered that the unfavorable effect of geographically-based discrimination on mental health was larger among the highly educated than among the less educated in a stratified analysis (Tabuchi et al., 2012). In Japan, a culture which historically values outward-facing status, high-SES individuals may have more resources to seek help, particularly if they live in urban areas where professionals (e.g., doctors, psychologists) are available. However, this trend might contribute to the stereotype that lower-SES citizens are less likely to seek help for mental illnesses, potentially out of choice (Poore et al., 2002). These social judgments seem to be more exaggerated in the U.S. than in Japan (Hanibuchi et al.,

2012; Kagamimori et al., 2009), although further research seems warranted.

Willingness To Seek Professional Help

Despite the stigma, seeking professional help to decrease symptoms of mental illness is beneficial (MentalHealth.gov, 2022). This means it is important to understand how the variables described above, when combined with cultural influence, make people less likely to seek help. Over half of adults with mental illnesses in the United States do not receive treatment, and 24.7 percent of adults with mental illnesses in 2019 reported an unmet treatment need (Mental Health America, n.d.). There are also significant differences in mental health treatment for adolescents of color, with White youth being the most likely to obtain therapy and Asian-American youth being the least likely to receive treatment (Mental Health America, n.d.), potentially reflecting attitudes from subcultures.

In Japan, there is a similar problem with underuse of professional psychological and psychiatric treatment (e.g., Follmer & Jones, 2018; Furukawa et al., 2000; Naito et al., 2020; Tajima, 2001). According to Munakata (1986), the societal expectation in Japanese culture is that people with diseases supposedly linked with a lack of mental or behavioral self-control will be supervised or cared for privately, by their family. As a result, mental illness is not seen as requiring professional medical attention.

Researchers have investigated a number of factors that might be linked to seeking help in order to better understand how people use psychological services. The attitudes of people about obtaining professional psychological help are one such influence. In the United States, several studies have investigated the nature of attitudes about getting psychiatric care (many of which are cited above), but there have been few in Japan—perhaps because seeking treatment for personal difficulties is traditionally seen as bringing dishonor to the family (Braun & Browne, 1998). According to Fukuhara (1986), Japanese college students suffering from psychological issues sought aid from friends and had unfavorable opinions regarding counseling. Negative attitudes regarding obtaining professional psychological help might also be linked to a lack of counseling information as well as a cultural stigma connected with self-disclosure (Mojaverian et al., 2013).

Hypotheses

The primary goal of this study was to explore if mental health stigma, locus of control, and SES predict an individual's attitude toward mental health and desire to seek professional help if needed. We also explored whether these variables differed by culture. We hypothesized that (1) higher mental illness stigma, higher internal locus of control, and lower SES would be associated with less likelihood of seeking professional help for mental illnesses and that (2) these associations would show different patterns in the U.S. versus in Japan.

Method

Participants

A total of 274 college students (130 men, 139 women, 5 others) participated. All were university students; 144 were from a midwestern U.S. university and 130 were from a rural university in Japan. Each university has about the same number of students on the relevant campus (a little fewer than 1000) and both are located in small, rural areas with towns of equivalent size (between 10,000-15,000 residents). See Table 1 for a summary of the demographics in each sample.

Table 1
Demographics by Country

	U.S.	Japan
	Gender	
Men	43.75%	51.94%
Women	54.86%	46.51%
Other	1.39%	1.55%
	Race	
White	81.25%	0%
Black	4.17%	0%
Hispanic	11.18%	0%
Asian	0%	100%
Other	2.78%	0%
	Age	
	M = 19.49	M = 19.17
	SD = 1.17	SD = 1.08

Measures

Rotter's (1966) scale was used to determine locus of control orientation. This is a self-report scale with 29 items, six of which are filler items. Filler items are often used in self-report surveys as a way to mask the purpose of the scale, to avoid demand characteristics in which participants attempt to manipulate their answers to either make themselves look more socially desirable or to answer in ways they think will please the research team (Rotter, 1966). Responses are in forced-choice format where participants have to choose between option A or option B. For example, (A), "In my case getting what I want has little or nothing to do with luck" (indicating an internal locus of control) versus (B), "Many times we might just as well decide what to do by flipping a coin" (indicating an external locus of control). An example filler item is (A), "There is too much emphasis on athletics in

high school" versus (B), "Team sports are an excellent way to build character." All answers connected to external locus of control receive one point, whereas those related to internal locus of control receive zero points. Composite scores can vary from 0-23, with higher scores indicating more of an external locus.

A modified version of the Kuppuswamy scale (Saleem & Jan, 2021) measured SES. It includes three broad domains of SES: occupation, total family income, and education. Items were modified to be in local currency (either USD or Japanese Yen) and to reflect modern typical salaries. For each domain, participants indicated their family history on a range from 1-6, with higher numbers indicating more education, status at work, and income. The composite SES score was a sum of the three responses, with a possible range from 3-21; higher scores indicate higher SES (Saleem & Jan, 2021). Internal consistency was good, $\alpha = .63$.

Stigma was measured using the Stigma Scale for Receiving Professional Psychological Help (Komiya et al., 2000). It consists of five items measuring one's perceived stigma about mental health treatment, such as, "It is a sign of personal weakness or inadequacy to see a psychologist for emotional or interpersonal problems." Responses for each range from 0 (*Disagree*) to 3 (*Agree*). Responses are averaged, leading to a possible range from 0-3, with higher scores indicating greater perception of stigma associated with receiving psychological treatment. Internal consistency was good, $\alpha = .62$.

Willingness to seek help was measured using the revised Attitudes Toward Seeking Professional Psychological Help Scale – Short Form (Fischer and Farina, 1995). It consists of 10 items, five of which are reverse scored. Participants read statements such as, "A person coping without professional help is admirable" and indicated their degree of agreement using a 4-point scale ranging from 1 (*Disagree*) to 4 (*Agree*). After reverse scoring, items ratings were averaged to create a composite score with a possible range from 1-4, where higher scores indicate more positive attitudes and willingness to seeking professional help. Internal consistency was good, $\alpha = .81$.

Procedure

In the United States, the English version of the scales was utilized by the first and second authors. The third author of this study is lecturer at a western university in Japan who is proficient in both English and Japanese. She produced the Japanese translation, which was checked and verified by a third party. Both groups of participants used an online software program (hosted by PsychData.com) to complete the questionnaire materials in their language. Participants from each university were recruited in classes taught by professors who had given permission for the research team to approach their students. Depending on the specific professor's preference, some students were directly sent emails with initial information and a link to the study's consent form. Other students heard about the study from a researcher who visited classes and spoke to the students as a group, then passed around sign-up sheets for anyone who wished to receive the email with the same link.

Either way, the link led students to the electronic survey on PsychData.com. Before beginning the survey questions, participants read the informed consent information and had to click a link to proceed; they could choose to stop participation at any time or skip any questions. They first provided demographic information, then completed all scales in the order described above. All participants were given the chance to win one of four $50 Amazon gift cards, and some U.S. participants also received extra credit. In order to preserve their anonymity, at the end of the survey students who wanted extra credit or to be entered in the lottery could click on a link taking them to a second survey where they could provide their name and contact information; this information was kept separate from the answers they provided on the research survey itself. The U.S. university's IRB approved this study and the Japanese university's administration provided equivalent letters of approval.

Results

Correlations

Hypothesis 1 was that higher mental illness stigma, higher internal locus of control, and lower SES would be associated with less likelihood of seeking professional help for mental illnesses. For exploratory purposes, we also calculated a correlation to test the association between gender and help seeking by dummy-coding participants with 1 for men and 2 for women. Three sets of correlations were completed: (1) for all participants combined, (2) for just participants in the U.S., and (3) for just participants in Japan.

Across all participants, professional help seeking was negatively correlated with mental illness stigma, $r(274) = -.20$, $p < .001$ (as predicted), but was positively correlated with external locus of control (in the opposite direction of our hypothesis), $r(274) = .22$, $p < .001$. It was not correlated with SES, $r(273) = -.01$, $p = .85$. We also found that help seeking was correlated with gender; women were more likely to indicate wiliness to seek help than men were, $r(273) = .30$, $p < .001$. Hypothesis 1 was therefore partially supported: Help seeking was more likely in individuals who have a low mental health stigma (as predicted). However, it was not correlated with SES

and, surprisingly, was positively correlated with higher external locus of control. Help seeking was also more likely in women.

The results split by country are shown in Table 2; numbers above the diagonal are for the U.S. sample and numbers below the diagonal are for the Japan sample. As shown in the first row, the U.S. sample showed the same pattern as that in all participants: help seeking was negatively correlated with stigma [$r(144) = -0.21$, $p = .01$] and positively correlated with higher external locus of control [$r(144) = .28$, $p < .001$) and with being a woman [$r(144) = .47$, $p < .001$]. However, the pattern was different in Japan. As shown in the first column of Table 2, the only variable significantly correlated with help seeking in those participants was mental illness stigma, $r(130) = -.30$, $p < .001$. All other correlations were not significant (all *p*s > .11). The differential pattern of results in the two countries supported Hypothesis 2.

Direct Country Comparisons

It is possible different patterns of correlations were found because base levels of help seeking differed by country. To check for this possibility, a series of *t*-tests compared the U.S. and Japanese samples on all variables of interest (see Table 3). Likelihood of help seeking was equivalent in the U.S. and Japanese samples, $t(272) = .83$, $p = .408$. That said, the other variables did have significant differences by country. People in the U.S. had significantly higher SES [$t(254) = 8.69$, $p < .001$] and higher stigma toward mental illness [$t(272) = 7.76$, $p < .001$], as well as significantly lower external locus of control [$t(272) = -2.21$, $p = .028$].

Regression

Across the entire sample, stigma, locus of control, and gender were all associated with different levels of help seeking. But as shown in Table 2, there are also correlations among the predictor variables in the U.S., with women making significantly less than men and with significantly higher scores on external locus of control. Therefore, a regression analysis was completed just for U.S. participants in which stigma, locus of control, and gender were all tested as simultaneous predictors of help seeking. Locus of control dropped out of significance ($p = .07$), while stigma ($p = .004$) and gender ($p < .001$) remained significant.

Table 2
Correlations Among Variables, By Country

Variable	1	2	3	4	5
1. Help Seeking	--	-.09	.28**	-.21*	.47**
2. SES	.07	--	-.10	.07	-.12
3. Locus of Control	.14	.00	--	.00	.34**
4. Stigma	-.30**	-.06	.08	--	.00
5. Gender	.08	.07	.05	-0.05	--

Note. Results from the U.S. sample are shown above the diagonal; results from the Japanese sample are shown below the diagonal. Degrees of freedom in the U.S. sample are always 144; degrees of freedom in the Japan sample range from 128-130.
*indicates $p < .05$; ** indicates $p < .01$.

Table 3
Comparisons by Country

	U.S. Sample		Japanese Sample			
	M	*SD*	*M*	*SD*	*t*	*p*
SES	11.71	2.76	9.24	1.89	8.69	< .001
LOC	12.58	3.81	13.44	2.60	-2.21	.028
Stigma	2.05	0.53	1.56	0.53	7.76	< .001
Help seeking	2.46	0.53	2.41	0.45	0.83	0.408

Note. LOC = Locus of control.

Discussion

This study explored if culture-based mental health stigma, locus of control, and socioeconomic status are substantially linked to an individual's attitude toward mental health and desire to seek professional help. Across all participants, professional help seeking was negatively correlated with mental illness stigma (as hypothesized) and positively correlated with an external locus of control (in the opposite direction of the hypothesis). However, it was not correlated with SES. While not specifically hypothesized, help seeking was also correlated with gender; women were more likely to indicate a willingness to seek help than men were. Hypothesis 2 was supported by the differential patterns of results in the two countries: The U.S. sample showed the same pattern as all participants; however, the only variable significantly correlated with help seeking in the Japanese sample was mental illness stigma.

These findings are similar to the findings of numerous previous studies highlighting the importance of mental health stigma regarding willingness to seek help. For example, Eisenberg et al. (2009) discovered that perceived stigma was a substantial barrier to getting care for mental health problems among college students in the United States. Similarly, Corrigan and Watson (2002) discovered that perceived stigma was a strong predictor of help seeking intentions among patients with mental illnesses.

While we expected help seeking to be associated with higher internal locus of control, the opposite occurred (at least, in the U.S. sample). Past work (e.g., April et al., 2012) has found that internal locus of control predicts greater wellbeing and less mental illness stigma. However, there are reasonable explanations for the opposite effect, as found here. Mackenzie et al. (2004) discovered that those with an external locus of control were more likely to seek care for mental health difficulties than those with an internal locus of control. Perhaps those with an external locus perceive that while they, personally, cannot control their mental health, it may be possible that other resources – such as a professional counselor or medication – may be effective, motivating them to seek help. Further research is clearly needed to fully understand the role of locus of control in this context, given the mixed results of studies so far.

The finding that help seeking was not associated with socioeconomic status fails to support some earlier research. For example, Mojtabai (2010) discovered that those with higher income and education levels were more likely to obtain treatment for mental health problems than those with lower income and education levels. However, it is possible that the sample used in the current study was not representative of the broader population, which may explain this discrepancy. The sample in this study did not include particularly high or low SES participants, therefore missing these individuals (i.e., a restricted range). More diversity of SES would be informative to further explore these connections.

The finding that women are more willing to seek help for mental health issues than men is consistent with previous research. For example, a study by Vogel et al. (2007) found that men were more likely to endorse stigmatizing attitudes toward mental illness and less likely to seek help for mental health issues than women. The differential pattern of results between the U.S. and Japanese samples in the current study is also interesting. The fact that only mental illness stigma was significantly correlated with help seeking in the Japanese sample may reflect cultural differences in how mental illness is perceived and stigmatized in Japan compared to the United States (in other words, in Japan national culture seemed to be more predictive than gender-based culture or any of the other constructs tested in this study).

Importance of Stigma

Taking on mental illness stigma is critical for improving mental health and encouraging help-seeking behavior. Research has demonstrated that stigma may result in embarrassment, shame, and fear of discrimination, which can deter people from getting treatment (Clement et al., 2015; Corrigan et al., 2002; Vogel et al., 2006). As shown by the current study's findings, stigma toward mental illness is associated with less help-seeking behavior, making it a significant factor. While causal implications cannot be made with correlational data, it is possible that the stigma associated with mental illness may impede getting care, harming one's mental health. Those who do not seek assistance cannot get the support they need to manage their symptoms and enhance their well-being, which can have significant consequences for mental health outcomes. Society and/or individuals might take numerous approaches to reduce stigma in the context of mental health.

To start, awareness and education about mental health can be increased, to lessen the misunderstandings and misinformation that frequently fuel stigmatization. This may entail efforts designed to clear up misconceptions and promote awareness of the causes and symptoms of mental illness. Second, more welcoming and encouraging settings might be created, encouraging individuals to ask for assistance when needed. This entails setting up places where people can discuss mental health difficulties without worrying about prejudice or judgment.

Finally, the media and popular culture can support accurate depictions of mental illness. This might entail encouraging sensitive and realistic depictions of psychological disorders in television programs, motion pictures, and other forms of media as well as publicizing the experiences of people who have successfully sought care for mental health difficulties. Lowering the stigma associated with mental illness is essential for encouraging behavior that leads to help-seeking and enhancing mental health outcomes. Finally, a more welcoming and encouraging environment can be fostered that encourages individuals to seek assistance, which may positively impact both the well-being of the person and the community by taking action to combat stigma.

Limitations & Strengths

These findings are somewhat consistent with past research on help-seeking behavior and are supported by studies such as those done by Corrigan and Watson (2002) emphasizing the central role of stigma. However, it is critical to address potential sources of bias and threats to internal and statistical validity that may alter the interpretation of these data. One potential cause is selection bias, which arises when participants are not representative of the investigated population—the sample for this study comprised college students from the United States and Japan. The findings may not apply to different populations or age ranges. Also, persons with poor mental health may have been more inclined to decline participation. The study might have included a more varied sample that better represented the larger community to increase the generalizability of the results.

Another possible source of bias is the accuracy of the measurement methods utilized in this study. There may have been measurement errors because self-report measures were used, or the survey questions might not have accurately represented the examined variables. For example, participants may have been more or less likely to report mental health stigma, external locus of control, or willingness to seek help due to social desirability bias or other factors. Future studies might employ various techniques to measure the relevant dimensions, such as interviews or behavioral measurements, to reduce these sources of bias.

The reported study's strengths include its relatively large sample size and use of numerous measures to investigate the association between help-seeking behavior and diverse predictors such as mental illness stigma, external locus of control, and gender. The study also included a cross-cultural approach, comparing data from individuals in the United States and Japan, which can give insight into the cultural characteristics that impact help-seeking behavior.

Future research can build upon this study's foundation by further exploring the questions and patterns of results found here. For example, other studies could use more precise indicators of help-seeking behavior, such as administrative data or medical records. In order to increase the generalizability of the results, future research may also involve a larger, more varied sample of participants, including people from various socioeconomic and age categories and including people from additional cultures and countries. Future research may also look at people's specific assistance-seeking strategies, such as approaching experts, acquaintances, or family members for help as well as distinguishing between people who seek talking forms of therapy versus biological or pharmaceutical treatments. This may provide insight into the specific factors that influence diverse assistance-seeking behaviors, which may also help develop targeted remedies.

The effectiveness of methods for promoting mental and physical health may vary across cultures. In Asian cultural contexts, seeking professional help may not align with the prevalent social interactions and relationships model, but this does not necessarily indicate a problem. By examining the link between seeking social support and seeking professional help and considering how culture influences coping behaviors, we can aim to gain a deeper understanding of and destigmatize the low rates of professional help seeking in Asian cultures (e.g., Fukuhara, 1986).

Implications

The reported findings have several implications for future research, programs, or policies to promote help-seeking behaviors and reduce mental illness stigma. First, the finding that mental illness stigma negatively correlates with help-seeking behavior suggests that interventions that reduce stigma might effectively promote help-seeking among college students (although again, this claim cannot be made without experimental results, due to lack of causal inference). Future research could explore the most effective ways to reduce stigma around mental illness and assess the impact of these interventions on help-seeking behavior. Programs or policies that promote awareness and education about mental health and that foster a supportive and inclusive environment may effectively reduce mental illness stigma and increase help-seeking behavior among college students.

The finding that external locus of control positively correlates with help-seeking behavior suggests that interventions should concentrate on encouraging individuals to attribute their successes and failures to external factors rather than internal factors; this may

help people avoid feeling blame when suffering from mental illness symptoms. Focusing on external factors (e.g., powerful others who can provide help and resources that can change outcomes in the future) might also encourage individuals to reach out to others when they need it, which could additionally promote help-seeking. Future research could also explore the nuances between when internal versus external locus of control will be predictive of mental health stigma and help-seeking when mediated and moderated by other factors beyond those explored in the current study.

Additionally, future research could explore the reasons for the gender differences in help-seeking behavior (we saw in the study that women were more likely to seek help) and develop tailored interventions that address the specific needs of men, women, and other genders. Different patterns of results between the U.S. and Japanese samples highlight the importance of considering cultural differences when designing interventions to promote help-seeking behavior. Future research could explore the cultural factors associated with help-seeking behavior and develop interventions tailored to specific cultural contexts.

Conclusion

Stigma against mental health appears to be a clear predictor of lower likelihood to seek help. The reported findings have important implications for future research, programs, or policies to promote help-seeking behavior and reduce mental illness stigma among college students. By addressing the specific needs and cultural contexts of college students, interventions aimed at promoting help-seeking behavior might impact the mental health and well-being of this population. Future research could also address these limitations by using more precise measurement protocols, adjusting for multiple comparisons, and improving the generalizability of the study. By carefully considering these limitations and working to overcome them, researchers can contribute to a deeper understanding of the similarities and differences between cultures when it comes to seeking help for mental illness.

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